



DOI_Z 50 MG FC TABLET

PRODUCT NAME

DOI_Z 50 MG FC TABLET

NAME AND STRENGTH OF ACTIVE SUBSTANCE

Each film-coated tablet contains 50 mg itopride hydrochloride

PRODUCT DESCRIPTION

A white to off-white, round biconvex film-coated tablet engraved with "A" logo on one side and "I" on the other side

PHARMACODYNAMICS

Pharmacotherapeutic group: gastrokinetic agent (a substituted benzamide)

Mechanism of action

Itopride has dual mechanism of action: dopamine D₂ receptor antagonism and acetylcholinesterase inhibition. The effect of itopride is an increase in acetylcholine concentration, which promotes gastric motility, increases the lower oesophageal sphincter (LOS) pressure, accelerates gastric emptying and improves gastroduodenal coordination. Itopride hydrochloride also has antiemetic action through interaction with D₂ receptors located in the chemoreceptor trigger zone. This was demonstrated by dose dependent inhibition of apomorphine-induced vomiting.

The action of itopride hydrochloride is highly specific for the upper gastrointestinal tract. Itopride hydrochloride does not affect serum gastrin levels.

PHARMACOKINETICS

Absorption

Itopride hydrochloride is rapidly and almost completely absorbed from the gastrointestinal tract. Relative bioavailability is calculated to be 60% due to liver first pass metabolism. There is no effect of food on bioavailability. Peak plasma levels (C_{max} = 0.28 g/mL) are reached within 0.5 to 0.75 hours following oral administration of 50 mg itopride hydrochloride.

Following multiple oral doses ranging from 50 mg to 200 mg TDS (three times daily), itopride hydrochloride and its metabolites showed linear pharmacokinetics over a treatment period of seven days, with minimal accumulation.

Distribution

Approximately 96% of itopride hydrochloride is bound to plasma proteins. Albumin accounts for most of the binding. Alpha-1-acid-glycoprotein accounts for less than 15% of binding.

Metabolism

Itopride undergoes extensive hepatic metabolism in humans. Three metabolites have been identified, of which only one exerts minor activity without pharmacological relevance (approximately 2-3% of that of itopride). The primary metabolite in humans is the N-oxide, generated by oxidation of the tertiary amine N-dimethyl group.

Itopride is metabolized by a flavin-dependent monooxygenase (FMO3). The abundance and efficiency of the human FMO-isozymes can be subject to genetic polymorphisms, which can lead to a rare autosomal recessive condition known as trimethylaminuria (fish odor syndrome).

Pharmacokinetics may differ between trimethylaminuria (TMAU) patients and non-trimethylaminuria (non-TMAU).

Excretion

Itopride hydrochloride and its metabolites are primarily excreted in the urine. The urinary excretions of itopride and its N-oxide were 3.7% and 75.4% respectively, after oral administration of a single therapeutic dose (50 mg).

The terminal phase half-life of itopride hydrochloride was approximately six hours.

INDICATION

Treatment of gastrointestinal symptoms caused by gastric dysmotility and delayed gastric emptying, like sensation of bloating, early satiety, postprandial fullness, upper abdominal pain or discomfort, anorexia, heartburn, nausea and vomiting; functional (non-ulcer) dyspepsia or chronic gastritis.

DOSAGE AND ADMINISTRATION

Method of administration

For oral use only

Adults

150 mg a day [one tablet (50 mg) taken three times a day before meals].

The dose may be reduced according to the patient's age and symptoms (see **Warnings and Precautions**).

Duration of treatment

Itopride hydrochloride is administered up to 8 weeks.

CONTRAINDICATIONS

Itopride hydrochloride is contraindicated in circumstances as below:-

- Patients with known hypersensitivity to itopride hydrochloride or any of the excipients
- Patients in whom an increase in gastrointestinal motility could be harmful (e.g. gastrointestinal hemorrhage, mechanical obstruction or perforation)

WARNINGS AND PRECAUTIONS

General

Itopride hydrochloride enhances the action of acetylcholine and may produce cholinergic side effects. Data on long-term use are not available.

Pediatric population

Safety of itopride hydrochloride in children under the age of 16 has not been established.

Geriatric population

In general, appropriate caution should be exercised in the administration and monitoring of itopride hydrochloride in elderly patients reflecting the greater frequency of decreased hepatic, renal function and of concomitant disease or other drug therapy.

INTERACTIONS WITH OTHER MEDICATIONS

- Metabolic interactions are not expected since itopride is primarily metabolized by flavine monooxygenase and not by CYP450
- No changes in protein binding have been seen with co-administration of warfarin, diazepam, diclofenac sodium, ticlopidine hydrochloride, nifedipine, and nicardipine hydrochloride

- Since itopride has gastrokinetic effects, it could influence the absorption of concomitant administration of oral drugs. Particular caution should be taken on drugs with a narrow therapeutic index, sustained release and enteric-coated formulations
- Anti-ulcer drugs like cimetidine, ranitidine, teprenone and cetraxate do not affect the prokinetic action of itopride
- Anticholinergic drugs may reduce the action of itopride

PREGNANCY AND LACTATION

Pregnancy

There are no or limited amount of data from the use if itopride in pregnant women. As a precautionary measure, it is preferable to avoid the use of itopride during pregnancy.

Lactation

Itopride is excreted in breast milk of animals, but there is insufficient information on the excretion of itopride in human milk. A risk to the suckling child cannot be excluded.

A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from itopride therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Although no effects on ability to drive and use machines have been found, impairment of alertness cannot be ruled out since dizziness may occur very rarely.

In these cases, the patient should not drive or use machines until symptoms disappear.

UNDESIRABLE EFFECTS

The following adverse reactions are classified according to system organ.

System Organ	Frequency	Adverse Reactions
Gastrointestinal disorders	Common Uncommon	: Diarrhea, abdominal pain : Salivary hypersecretion
Investigations	Uncommon	: Alanine aminotransferase increased, white blood cell count decreased
Nervous system disorders	Uncommon	: Dizziness, headache
Skin and subcutaneous tissue disorders	Uncommon	: Rash
Blood and lymphatic system disorders	Cannot be estimated	: <u>Post-marketing experience</u> : Leukopenia, thrombocytopenia
Hepato-biliary disorders	Cannot be estimated	: <u>Post-marketing experience</u> : Jaundice
Reproductive system and breast disorders	Cannot be estimated	: <u>Post-marketing experience</u> : Gynecomastia

OVERDOSE AND TREATMENT

In case of excessive overdose, the usual measures of gastric lavage and symptomatic therapy should be applied.

STORAGE CONDITIONS

Store below 30°C in the original package.

Itopride hydrochloride should be protected from light and moisture.

SHELF LIFE

Refer to unit box for shelf-life.

DOSAGE FORMS AND PACKAGING AVAILABLE

1 blister strip x 10 film-coated tablets per unit box

6 blister strip x 10 film-coated tablets per unit box

DATE OF REVISION OF PI

Date of revision: 14/03/2025

Manufacturer:- SIAM BHEASACH CO., LTD.

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